🧠 Psychiatrist Appointment — Preparation Sheet

> **Purpose:** Help you walk into your first psychiatric consultation with a clear, organized picture of what's happening — so nothing important gets lost in the moment. Fill in or edit the bracketed `[...]` sections before you go.

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1. One-Sentence Summary for the Doctor

> *"For about 7 years — since the start of the COVID pandemic — I've had serious, escalating health problems that have made it difficult to stay alive. I'm dealing with active ulcerative colitis, a recent food poisoning episode, and cognitive deficits (brain fog, fluctuating focus, mental fatigue). I believe these are connected and I need help understanding what's driving what, and what treatment options are available."*

Adjust this to match your actual experience. Having a clear opening sentence helps the doctor orient quickly.

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2. Your Main Concerns (In Priority Order)

List what matters most to you. Rank them so the doctor knows where to focus if time is short.

#	Concern	How long?	Impact on daily life (1–10)
1	Serious issues staying alive	~7 years (since COVID)	[10?]
2	Cognitive deficits (brain fog, focus, memory)	[since when?]	[?]
3	Active ulcerative colitis	[diagnosed when?]	[?]
4	Food poisoning episode	[when did it happen?]	[?]
5	Symptom fluctuation / unpredictability	[ongoing?]	[?]
6	Can't find safe food — most supermarket food triggers symptoms	[ongoing?]	[?]
7	Can't access brain-recovery supplements locally	[ongoing?]	[?]

> ⚠️ **"Staying alive" matters.** If you mean you've been struggling with suicidal thoughts, hopelessness, or feeling like your health is actively threatening your survival — **tell the psychiatrist directly**. This is the most important thing you can communicate. They need to know the full severity to help you safely.

2b. Practical Barriers You're Facing (Bring These Up)

These aren't side complaints — they directly affect your health and your ability to follow any treatment plan. The psychiatrist needs to know about them.

🍎 Food Is Working Against You

> *"Most food available in the supermarket makes my UC worse. I struggle to find anything safe to eat. This isn't a preference — it's a daily barrier to staying alive."*

Fill in before the appointment:

- Which foods reliably trigger your UC? (list them)
- What *can* you eat safely? (list the safe foods — even if the list is short)
- Have you tried any specific diets? (SCD, low-FODMAP, elimination, autoimmune protocol, etc.) What happened?
- Have you worked with a dietitian? If not — ask for a referral
- How much weight have you lost (if any)?
- Are you malnourished or borderline? (this matters for brain function)

> 💡 **Why this matters for the psychiatrist:** Malnutrition from a restrictive UC diet can *directly cause* cognitive deficits (iron, B12, folate, omega-3 deficiency). The "brain fog" you're experiencing may be your brain literally starving for nutrients. Tell them you can't fix the brain until you can fix the food intake.

💊 You Can't Access Brain-Recovery Supplements

> *"I've researched what might help my brain recover, but I can't find these supplements anywhere locally. It feels like the tools I need aren't available to me."*

Fill in before the appointment:

- Which supplements have you tried to find but couldn't? (list them)
- Which ones did you find but were too expensive, out of stock, or questionable quality?
- Have you ordered online? What happened? (shipping restrictions, customs, quality concerns)
- What does your pharmacy carry vs. not carry?

Questions to ask the psychiatrist about this:

- ☐ *"I can't access certain neurological supplements locally. Are there prescription alternatives that cover the same pathways?"* (e.g., prescription cholinesterase inhibitors, NAD+ precursors, anti-inflammatories)
- ☐ *"Can you prescribe something that addresses the cognitive deficits directly while we work on the underlying causes?"*
- ☐ *"Are there hospital pharmacy or specialty pharmacy channels that carry things regular pharmacies don't?"*
- ☐ *"Do you have patients in similar situations — what do they do for supplement access?"*
- ☐ *"Could I get a referral to a neurologist or cognitive rehabilitation specialist who might have more options?"*

> ⚠️ **Be honest about the frustration.** Say: *"I've been trying to help myself for 7 years. I've done the research. But the tools I need aren't available to me, and I'm running out of options. I need help accessing actual treatment, not just advice."*

2c. Neuroplasticity — What I Need and Why

> *"I'm cognitively challenged and emotionally unstable. I can't keep memory or a stable personality. I need something that helps neuroplasticity — and I need it to help soon."*

What I'm Starting (Available Without Prescription)

Supplement	Target	Sourcing
Omega-3 (high EPA/DHA)	↑ BDNF, ↓ neuroinflammation	Pharmacy — same day
Creatine monohydrate (5 g/day)	↑ brain ATP, ↑ cognitive recovery	Supplement shop — same day
Magnesium L-threonate	↑ synapse density, crosses BBB	EU online — 3–5 days
Lion's Mane (dual extract)	↑ NGF, ↑ neurogenesis	EU online — 5–7 days
NAC	↑ glutathione, ↓ neurotoxicity	Pharmacy/EU online
Curcumin + piperine	↑ BDNF, ↓ inflammation (also helps UC)	Pharmacy — same day
Bacopa monnieri	↑ BDNF, ↑ memory (takes 4–6 weeks)	EU online — 5–7 days

What I'm Asking the Psychiatrist to Consider Prescribing

> ⚠️ **Important:** These are options to *discuss*, not demands. Frame it as: *"I've researched these — can you tell me which might be appropriate for my situation?"* The psychiatrist will decide based on your full clinical picture, interactions with UC medications, and safety.

🧠 Neuroplasticity & Cognitive Recovery

Medication	How it works	Why it's relevant for you	UC safety note
Vortioxetine (Brintellix)	SSRI + direct ↑ neuroplasticity, ↑ cognition, ↑ BDNF	Targets BOTH mood AND cognitive deficits in one pill — may be the single best option for your combination	Generally gut-safe; lower GI side effects than typical SSRIs
Low-dose lithium (lithium orotate or prescription)	↑ BDNF, ↑ neurogenesis, ↑ emotional stabilization, ↓ impulsivity	Specifically relevant to "unstable personality" / mood swings / identity instability	Monitor kidney function — lithium can affect kidneys long-term
Memantine	NMDA antagonist → ↑ BDNF, ↑ glutamate modulation, ↑ plasticity	Used off-label for cognitive recovery, brain fog, neuroprotection	Generally gut-safe
Fluoxetine (Prozac)	SSRI + ↑ BDNF, ↑ neurogenesis in hippocampus	Long track record for promoting neuroplasticity; activating	Can cause nausea/GI upset initially — start low

⚡ Immediate Cognitive Support (If Function Is Severely Impaired)

Medication	How it works	Why it's relevant	UC safety note
Methylphenidate (Ritalin/Concerta)	↑ dopamine, ↑ norepinephrine	Immediate focus, working memory, task initiation — can function within hours	Generally gut-safe; may reduce appetite (could worsen your nutrition issue)
Modafinil / Armodafinil	↑ dopamine (via DAT), ↑ orexin, ↑ histamine		Generally gut-safe; prescription-only in SI

Medication	How it works	Why it's relevant	UC safety note
		Wakefulness, sustained focus, reduced fatigue — 10–12 hour effect	
Lisdexamfetamine (Elvanse)	Prodrug of amphetamine → ↑ dopamine, ↑ norepinephrine	Long-acting focus and cognitive support; smoother than immediate amphetamine	May reduce appetite; monitor nutrition

🛡️ Neuroprotection & Anti-Inflammatory (Targets UC → Brain Pathway)

Medication	How it works	Why it's relevant	UC safety note
Minocycline	Anti-inflammatory + neuroprotective (not just antibiotic)	Reduces microglial activation → ↓ neuroinflammation; studied for cognitive recovery	Can affect gut microbiome — use with probiotics
Naltrexone (low-dose / LDN)	Low-dose: ↑ endorphins, ↑ immune regulation, ↓ neuroinflammation	Off-label for autoimmune conditions and neuroinflammation; may help both UC and brain	Generally gut-safe; prescription needed
Pioglitazone	PPAR-γ agonist → ↓ neuroinflammation, ↑ mitochondrial function	Studied for neuroprotection and cognitive recovery	Monitor liver function

😔 Mood & Emotional Stabilization

Medication	How it works	Why it's relevant	UC safety note
Lamotrigine	Mood stabilizer → ↑ glutamate regulation, ↑ neuroplasticity	For mood swings, identity instability, emotional lability — without the weight gain of other mood stabilizers	Generally gut-safe
Quetiapine (low-dose Seroquel)	Atypical antipsychotic (low dose) → mood stabilization, sleep improvement	For severe mood instability, sleep disruption, emotional dysregulation	Can cause sedation, weight gain, metabolic effects
Valproate (Depakote)	Mood stabilizer → ↑ GABA, ↑ neuroprotection	For rapid mood swings, impulsivity, identity instability	Can affect liver and GI — monitor with UC
Mirtazapine (Remeron)	NaSSA → ↑ serotonin, ↑ norepinephrine, ↑ appetite	Dual benefit: mood + sleep + **increases appetite** (could help your nutrition problem)	Weight gain likely (could be beneficial for you); generally gut-safe

💉 Injectable / Advanced Options (Ask About Availability)

Medication	How it works	Why it's relevant	UC safety note
Esketamine (Spravato) nasal spray	NMDA antagonist → rapid ↑ BDNF, ↑ neuroplasticity	For treatment-resistant depression; rapid-acting (hours to days, not weeks)	Generally gut-safe; administered in clinic
B12 injections (if deficient)	Direct B12 bypassing gut absorption	If B12 is low from UC malabsorption, oral B12 won't work — you need injections	Safe; directly treats cognitive symptoms if deficient
Iron infusions (if deficient)	IV iron bypassing gut	If ferritin is low from UC bleeding, oral iron worsens UC — IV is safer and faster	Safe; can improve cognitive symptoms within weeks

Direct Script for the Doctor

> *"I need something that helps immediately. My cognitive function and emotional stability are declining to the point where I can't maintain memory or a consistent sense of self. I'm starting Omega-3, creatine, magnesium threonate, and Lion's Mane on my own. But I need something prescription-strength that targets neuroplasticity directly — I've read about vortioxetine, low-dose lithium, memantine, and possibly methylphenidate for immediate function. Would any of those be appropriate given my UC? What can you prescribe today? I also want to check if I need B12 or iron injections — my gut may not absorb oral forms."*

Key Questions About Prescriptions

- [ ] *"Which of these target neuroplasticity directly, not just symptoms?"*
- [ ] *"Given my active UC, which of these are safest for my gut?"*
- [ ] *"Could I start something today while we wait for bloodwork results?"*
- [ ] *"If I need B12 or iron, can I get injections instead of oral supplements?"*
- [ ] *"How quickly should I expect to feel improvement from each option?"*
- [ ] *"What's the plan if the first thing we try doesn't work?"*

Critical Symptoms to State Clearly

- [ ] **"I can't keep memory"** — specify: short-term? Long-term? Both? Give a recent example
- [ ] **"I can't keep a stable personality"** — this is important. Describe what this feels like:
  - Do you feel like different people at different times?
  - Do your values/opinions/identity shift dramatically?
  - Do you feel disconnected from yourself (depersonalization/derealization)?
  - Is it mood swings, or something deeper — like you don't recognize yourself?
  - *Write your description here:*
- [ ] **"I need something that helps immediately"** — say this directly. Don't downplay it.

2d. What "Unstable Personality & Memory Loss" Could Mean

This is not just brain fog. "Unable to keep memory or a stable personality" is a serious symptom that could point to several underlying causes — the psychiatrist needs to differentiate between them:

Possible Cause	Why It's Relevant	How It's Tested/Diagnosed
Chronic neuroinflammation from UC	Cytokines (IL-6, TNF-α) cross blood-brain barrier → neuroinflammation → cognitive + identity disruption	CRP, calprotectin, clinical assessment
Severe B12 / folate / iron deficiency (malabsorption from UC)	Can cause irreversible neurological damage, memory loss, mood/personality changes, depersonalization	**Bloodwork: ferritin, B12, folate, CBC** — do this BEFORE the appointment if possible
Dissociation / depersonalization	Chronic illness + 7 years of stress can trigger dissociative symptoms — feeling detached from self, identity shifts	Clinical psychiatric assessment
Neurological condition	Memory loss + personality instability can signal a neurological issue requiring a neurologist	Neurological exam, brain imaging, neuropsychological testing

Possible Cause	Why It's Relevant	How It's Tested/Diagnosed
Severe depression with pseudodementia	Depression can mimic dementia — memory loss, cognitive decline, personality changes	Psychiatric assessment, depression screening
Long COVID neurology	Post-COVID brain fog, identity disturbance, and cognitive decline are documented	Clinical history, exclusion of other causes

> ⚠️ **Tell the psychiatrist:** *"I'm not just foggy. I'm losing my ability to maintain memory and a stable personality. I need to know if this is neurological, psychiatric, inflammatory, or nutritional — because the treatment is completely different depending on the cause."*

> ⚠️ **Ask directly:** *"Should I be referred to a neurologist for this, or is psychiatry the right starting point? Could this be B12 deficiency brain damage from my UC?"*

2e. What You Can Do TODAY — Before the Appointment

Same-Day Actions

1. **Buy Omega-3 (high EPA/DHA)** — any Slovenian pharmacy. Safe with UC. Directly supports BDNF and neuroplasticity. Start with 1–2 g/day with meals.
2. **Buy Creatine monohydrate** — any supplement shop (SportNutrition, GymNutrition, etc.). Safe with UC. 5 g/day. Supports brain ATP and cognitive recovery.
3. **Write down 2–3 specific examples** of:
   - Memory loss: a specific moment you couldn't remember something you should have
   - Personality instability: a specific moment you felt like you weren't yourself
   - *Bring these written examples to the appointment — don't rely on memory in the moment*

Before the Appointment (If Time Allows)

1. **Get bloodwork from your GP** — ask for:
   - **Ferritin / iron panel** (iron deficiency → cognitive impairment, especially with UC bleeding)
   - **B12 and folate** (deficiency → memory loss, personality changes, neurological damage)
   - **Vitamin D** (deficiency → mood and cognition)
   - **CRP and calproprotecin** (inflammation markers — UC activity)
   - **Thyroid panel** (TSH, free T3, free T4 — thyroid dysfunction mimics psychiatric symptoms)
   - **Complete blood count** (anemia check)

   > If B12 is low, this could explain a large part of what you're experiencing — and it's treatable. Bring results to the psychiatrist.

2. **Order from EU online** (ships to Slovenia, 3–7 days):
   - Magnesium L-threonate (iHerb, Solgar)
   - Lion's Mane dual extract (Real Mushrooms, Nootropics Depot EU)
   - NAC (pharmacy or iHerb)
   - Curcumin + piperine (pharmacy or iHerb)

- Bacopa monnieri — Himalaya brand (iHerb, EU Amazon)

> ⚠️ **Do NOT start any new supplement the day before the appointment** — the psychiatrist needs to know what you're taking, and some supplements interact with psychiatric medications.

2f. FOOD That Heals Your Brain AND Is Safe With UC

> You said most supermarket food is poisonous for you. This is the cross-section: foods that **actively repair your brain** (↑ BDNF, ↑ neuroplasticity, ↑ neurotransmitters) **AND** are **safe for ulcerative colitis**. All available in Slovenia.

🐟 Tier 1: Eat These Daily — Brain + Gut Healers

Food	Brain benefit	UC safety	Where to buy in Slovenia	How to eat
Fatty fish (salmon, mackerel, sardines, herring)	Highest natural source of EPA/DHA omega-3 → ↑ BDNF, ↑ neuroplasticity, ↓ neuroinflammation	Recommended by IOIBD for UC — omega-3s reduce intestinal inflammation	Mercator, Spar, Lidl (fresh/frozen sardines & mackerel are cheap; salmon is pricier)	Baked or grilled, NOT fried. Sardines from a can are fine — pick ones in olive oil, not sunflower. 2–3x/week
Extra virgin olive oil	↑ BDNF, polyphenols ↓ neuroinflammation, ↑ cerebral blood flow	One of the FEW oils recommended for UC — small amounts may decrease UC risk	Any Slovenian supermarket — look for cold-pressed (hladno stiskano)	1–2 tablespoons daily, raw over food or used for low-heat cooking
Eggs (pastured if possible)	Richest source of **choline** (precursor to acetylcholine → memory, focus) + B12 + B6 + folate → directly repairs cognitive deficits	Generally well-tolerated in UC — easy to digest, nutrient-dense	Any supermarket, Mercator, local farms	Soft-boiled or scrambled. Avoid raw if UC is flaring. 2–4/day is safe
Blueberries	Anthocyanins cross blood-brain barrier → ↑ BDNF, ↑ memory, ↑ neurogenesis. One of the most studied brain foods	Generally UC-safe — well-tolerated by most. If raw causes issues, try cooked/blended	Mercator, Spar, Lidl, local markets (jagode)	Fresh or frozen. If raw bothers your gut, blend into a smoothie or cook into compote. 1 handful/day
Bone broth (goveja ali piščančja juha)	Collagen + amino acids (glycine, proline) → repair gut lining AND repair blood-brain barrier. Glycine → ↑ GABA → calms anxiety	One of the best foods for UC — heals intestinal lining, easy to digest, anti-inflammatory	Make at home from bones (kosti) from butcher (mesnica) — cheap or free	1 cup daily. Sip warm. Add salt. During flares, this may be one of the few things you can tolerate

🫀 Tier 2: Eat These Several Times a Week

Food	Brain benefit	UC safety	Where to buy	Notes
Liver (beef or chicken — goveja ali piščančja jetra)	The single most concentrated source of **B12, folate, iron, choline** → directly treats the deficiencies causing your cognitive symptoms	Generally UC-safe when cooked. Avoid if you have gout	Butcher (mesnica), Mercator	1–2x/week. Pan-fried with olive oil. If texture bothers you, blend into pâté. This is your B12 insurance if you can't get injections

Food	Brain benefit	UC safety	Where to buy	Notes
Walnuts (orehi)	Highest plant omega-3 (ALA) + polyphenols → ↑ BDNF, ↑ neuroplasticity	Recommended for UC by IOIBD — BUT some people can't tolerate nuts during flares. Test carefully. If whole nuts irritate, try walnut oil or finely ground	Any supermarket, local markets	If whole nuts trigger symptoms, grind into powder or use walnut oil. 1 small handful/ day IF tolerated
Dark chocolate (≥70% cocoa)	Cocoa flavanols ↑ BDNF, ↑ cerebral blood flow, ↑ memory	UC-safe in small amounts — BUT avoid if caffeine triggers your gut. Some people with UC report chocolate triggers symptoms. Test a small amount first	Mercator, Spar, Müller	1–2 small squares/ day. Start with a tiny amount to test UC tolerance
Turmeric (kurkuma)	Curcumin → ↑ BDNF, strongest natural anti-inflammatory, ↓ neuroinflammation, may help UC directly	Studied as a UC treatment — anti-inflammatory in the gut. Generally well-tolerated	Any supermarket, spice section (kurkuma v prahu)	1 tsp/day. Mix with black pepper (piperine increases absorption 20x) and olive oil. Add to bone broth, eggs, or rice
Pumpkin seeds (bučna semena)	High in **magnesium, zinc, iron, tryptophan** → ↑ serotonin, ↑ sleep, ↑ neuroplasticity. Tryptophan → serotonin → mood stabilization	Seeds can irritate UC during flares. If whole seeds are a problem, try pumpkin seed oil (bučno olje) — a Slovenian specialty	Any supermarket, local markets (Styrian pumpkin seed oil is Slovenian)	If seeds trigger UC, use bučno olje (pumpkin seed oil) — 1 tablespoon over food. Slovenian tradition, widely available
Spinach (špinaca) — cooked, not raw	Folate, iron, magnesium, B vitamins → directly supports neurotransmitter production and cognitive function	UC-safe when **well-cooked**. Raw spinach is high-fiber and may irritate. Always cook thoroughly	Any supermarket	Sauté in olive oil with garlic. Never raw during a flare. 2–3x/ week

🍚 Tier 3: Safe Carbs That Don't Irritate UC (Your Energy Base)

Food	Brain benefit	UC safety	Notes
White rice (beli riž)	Easy energy for brain (brain uses 20% of body's glucose)	One of the safest UC foods — low fiber, easy to digest	Add olive oil + turmeric + salt. Eat with fish or eggs
Oats (ovsena kaša) — well-cooked	Soluble fiber → feeds gut microbiome → ↑ short-chain fatty acids → ↑ BDNF. Contains B vitamins	Usually UC-safe when well-cooked. Avoid during severe flares. Start with small amounts	Cook thoroughly. Avoid raw oats or granola
Bananas (banane) — ripe	Potassium, B6, tryptophan → ↑ serotonin, ↑ sleep, easy energy	One of the safest UC foods — easy to digest, low fiber when ripe	Ripe only. Green bananas are harder to digest
Potatoes (krompir) — peeled, boiled/ mashed	Potassium, B6, vitamin C → supports neurotransmitter synthesis	UC-safe when peeled and boiled. Avoid fried (fries) or with skin	Mashed with olive oil (not butter if dairy-sensitive)
Applesauce (pasirana jabolka) — no skin	Pectin → feeds gut microbiome, soothes intestinal lining	UC-safe when peeled and cooked. Raw apples with skin may irritate	Buy or make at home — cook peeled apples, blend

🚫 Foods That Destroy Both Your Brain AND Your Gut — Avoid Completely

Food	Why it destroys your brain	Why it destroys your gut
Processed seed oils (sunflower, soybean, corn, canola)	High omega-6 → ↑ neuroinflammation → ↓ BDNF, ↓ cognition	Promotes intestinal inflammation, worsens UC
Refined sugar (sladkor)	Spikes insulin → crashes BDNF, damages hippocampus, causes brain fog	Feeds bad gut bacteria → dysbiosis → UC flares
Processed meats (salami, sausages, hot dogs)	Nitrates → neuroinflammation, oxidative stress	High fat + additives → UC trigger
Artificial sweeteners (aspartame, sucralose)	Aspartame → neurotoxicity, ↓ cognitive function	Alter gut microbiome → dysbiosis → UC
Fried foods	Oxidized fats → neuroinflammation, ↓ BDNF	Hard to digest → UC trigger, diarrhea
Alcohol	Directly toxic to brain → ↓ BDNF, kills neurons, disrupts sleep	Major UC trigger — damages gut lining
Spicy foods (if they trigger you)	Indirect — pain and inflammation stress the system	Common UC trigger during flares

📋 A Simple Day of Brain-Healing, UC-Safe Eating

Meal	What	Why
Morning	Bone broth (warm cup) + 2 soft-boiled eggs with turmeric + olive oil	B12, choline, collagen, curcumin, easy to digest
Snack	Banana + 1 square dark chocolate (if tolerated)	Potassium, B6, cocoa flavanols
Lunch	Baked salmon or sardines + white rice with olive oil + cooked spinach	Omega-3, BDNF, easy carbs, folate, iron
Snack	Blueberries (fresh or blended if raw irritates)	Anthocyanins, BDNF
Dinner	Liver (1–2x/week) or chicken + mashed potatoes + olive oil + cooked carrots	B12, iron, folate, easy to digest
Before bed	Pumpkin seed oil (1 tbsp) or handful walnuts (if tolerated)	Tryptophan, magnesium, serotonin → sleep

> 💡 **Tell the psychiatrist:** *"I've figured out which foods help my brain and are safe for my UC — fatty fish, eggs, bone broth, blueberries, olive oil, turmeric. But I'm struggling to maintain this diet consistently because of the cognitive impairment itself. I need support to sustain it — can you connect me with a dietitian who understands IBD?"*

Sources

- BDNF-boosting foods: Amos Institute, Jazz Psychiatry, npnHub, OrganiClinic
- UC-safe foods: Crohn's & Colitis Foundation, Cleveland Clinic, Medical News Today, Verywell Health, Mayo Clinic
- Ketogenic diet ↑ BDNF: PMC
- Carbohydrate-restricted diet ↑ BDNF in 4 weeks: PMC
- Olive oil ↓ UC risk: Verywell Health

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3. Symptom Tracking — Be Specific

Psychiatrists need **concrete examples**, not just labels. Fill this in before the appointment.

Cognitive Symptoms

- **Brain fog:** How does it feel? (e.g., "like thinking through mud", "can't hold a thought")
- **Memory:** Short-term? Long-term? Names? Tasks? *Give a specific recent example.*
- **Focus/attention:** Can you read a full article? Hold a conversation? Work for 30 min straight?
- **Processing speed:** Do you feel slower than usual? Give an example.
- **Word-finding:** Do you lose words mid-sentence?

Physical / Gut Symptoms (Relevant Context)

- **Ulcerative colitis status:** Currently in flare? What symptoms? (bleeding, diarrhea, pain, urgency)
- **Food poisoning details:** What happened? How severe? Did you seek medical care? Any lingering effects?
- **Nutrition:** Are you eating normally? Any food restrictions due to UC? Weight loss?
- **Sleep:** Hours per night? Quality? Do you wake up? Any connection between bad gut days and bad brain days?

Mood & Energy (Even If Not Your Main Focus — They'll Ask)

- **Mood:** Low? Anxious? Irritable? Flat? Does it fluctuate with your gut symptoms?
- **Energy:** Morning vs. evening? Crashes after eating?
- **Motivation:** Is it hard to start tasks, or hard to sustain them?
- **Anxiety:** Any panic, racing thoughts, or physical anxiety symptoms?

Pattern Observation

- Do your cognitive symptoms **track with** your UC flare-ups? (This is a key question — the gut-brain axis)
- Do they **track with** meals, sleep, stress, or supplement intake?
- Are there **good days and bad days**, or is it constant?
- What makes a **good day** different from a bad day?

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4. Complete Supplement & Substance List

> ⚠️ **This is critical.** Be completely honest. Psychiatrists need to know *everything* you take, including over-the-counter supplements, nootropics, and any recreational substances. They are not there to judge you — they need this to avoid dangerous interactions and to understand what's already in your system.

Current Supplements / Self-Medication

Substance	Dose	Frequency	How long?	Why did you start?	Effect?
[e.g. Nicotine gum/patch]	[?] mg	[?x/day]	[?]	[?]	[?]
[e.g. Alpha-GPC]					
[e.g. L-Theanine]					
[Add all supplements...]					

Past Supplements Tried & Stopped

Substance	Why did you stop?	Any withdrawal or side effects?
[e.g. Modafinil]	[?]	[?]
[Add others...]		

Other Substances

- **Caffeine:** How much? (coffee, tea, energy drinks, mg if known)
- **Alcohol:** How often? How much?
- **Nicotine:** Form? (smoking, vaping, gum, patch, snus) How much?
- **Cannabis / other:** Be honest — it matters for interactions
- **Any prescription meds from other doctors?** (GI meds, pain meds, antibiotics for food poisoning, etc.)

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5. Medical History Snapshot

Diagnoses

Timeline (Critical — Draw This for the Doctor)

- **~2019–2020 (COVID start):** This is when things started getting seriously bad. What happened? (COVID infection? Lockdown stress? Did UC start or worsen around this time? Did something else trigger the decline?)
- **2020–present (~7 years):** Ongoing, escalating health struggle. Periods of worsening vs. relative stability?
- **Ulcerative colitis:** When diagnosed? Which part of colon? Current severity?
- **Food poisoning:** When? What pathogen (if known)? Treated with antibiotics? Any lingering effects?
- **Other conditions:** [thyroid issues, anemia, vitamin deficiencies, autoimmune, COVID-related, etc.]

> 💡 **Tip:** Try to sketch out a rough timeline of what happened when. Even approximate dates help the psychiatrist see patterns. For example: *"2020 — COVID, health started declining. 2021 — UC diagnosed. 2023 — worst flare. 2024 — food poisoning. 2024–now — cognitive issues worsening."*

Specific Questions to Ask Yourself

- Did you have COVID-19? How severe? Any long COVID symptoms? (fatigue, brain fog, and GI issues are common long COVID symptoms — and they overlap heavily with UC and psychiatric symptoms)
- Was there a major life event around 2019–2020 beyond COVID itself? (loss, trauma, major stress)
- Did the health issues start *before* or *after* COVID?
- Has there been a clear progression (getting worse over time), or has it been more episodic?

Relevant Lab Results (Bring Copies If You Have Them)

- **Iron / ferritin** — low iron directly causes cognitive symptoms and fatigue (especially with UC bleeding)
- **B12 / folate** — UC and gut damage can impair absorption → cognitive deficits
- **Vitamin D** — deficiency linked to mood and cognition
- **CRP / calprotectin** — inflammation markers (UC activity)
- **Thyroid panel** — thyroid dysfunction mimics psychiatric symptoms

- **Complete blood count** — anemia check
- **Any other recent labs?**

> 💡 **Tip:** If you don't have recent labs, ask the psychiatrist whether they'd recommend ordering any of these. The cognitive symptoms could be nutritional deficiencies from malabsorption (common in active UC), not a primary psychiatric condition.

Medication History

- Any psychiatric medications tried before? (even briefly)
- Any adverse reactions or allergies?
- Current GI medications? (mesalamine, biologics, steroids, immunosuppressants)

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6. The Gut-Brain Connection — Key Talking Points

This is the core hypothesis you want to discuss. Here's the science-backed framework so you can explain it clearly:

How UC Can Cause Cognitive Symptoms

1. **Systemic inflammation:** Active UC releases pro-inflammatory cytokines (IL-6, TNF-α) that cross the blood-brain barrier → neuroinflammation → brain fog, fatigue, low mood
2. **Malabsorption → deficiencies:** Damaged gut lining = poor absorption of iron, B12, folate, vitamin D → all directly cause cognitive impairment
3. **Gut microbiome disruption:** Food poisoning + UC = dysbiosis → altered neurotransmitter production (90% of serotonin is made in the gut)
4. **Vagus nerve signaling:** Gut inflammation disrupts vagal communication to the brain → reduced cognitive function, mood changes
5. **Sleep disruption:** UC symptoms (pain, urgency, diarrhea) destroy sleep quality → cognitive deficits accumulate
6. **Steroid effects:** If you've taken prednisone/steroids for UC flares, these can cause mood swings, anxiety, insomnia, and cognitive changes

The COVID Factor (Don't Overlook This)

If your decline started around the COVID pandemic, this adds another layer:

- **Long COVID / post-COVID syndrome:** Brain fog, fatigue, GI disruption, and depression are among the most common long COVID symptoms. This could be a separate contributor overlapping with UC
- **Chronic inflammation from COVID:** SARS-CoV-2 can trigger persistent systemic inflammation and autoimmune flare-ups (UC is autoimmune — COVID may have worsened it)
- **Social isolation / lockdown impact:** 7 years of reduced social contact, disrupted routines, and stress can independently cause or worsen mental health
- **Gut-virus interaction:** COVID infects the gut (ACE2 receptors are highly expressed in the intestines) — this could have triggered or worsened UC

> 💡 **Ask the doctor:** *"Could long COVID be contributing to my symptoms alongside the UC? Is there overlap, and does it change the treatment approach?"*

What to Tell the Doctor

- *"I've noticed my cognitive symptoms seem to correlate with my UC flare-ups"*
- *"The food poisoning seemed to make things worse — is there a connection?"*
- *"Could this be inflammation-driven or nutritional deficiency rather than a primary psychiatric issue?"*
- *"I want to understand whether treating the gut inflammation could resolve the cognitive symptoms"*
- *"I'm open to psychiatric medication, but I want to make sure we're not treating a symptom of something else"*

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7. Questions to Ask the Psychiatrist

About Your Diagnosis

- ☐ Could my cognitive symptoms be caused by inflammation from my UC rather than a primary psychiatric condition?
- ☐ Should I get specific blood work done? (iron panel, B12, folate, vitamin D, thyroid, inflammatory markers)
- ☐ Could the food poisoning have triggered a longer-term gut-brain disruption?
- ☐ Is there a differential diagnosis I should be aware of? (e.g., delirium, encephalopathy, depression, ADHD, deficiency-induced cognitive impairment)

About Treatment

- ☐ What are the treatment options for inflammation-related cognitive impairment?
- ☐ Would anti-inflammatory approaches (e.g., addressing the UC flare) help the brain symptoms?
- ☐ Are there psychiatric medications that could help in the short term while the gut heals?
- ☐ Are any of my current supplements contraindicated or dangerous in combination with UC or potential medications?
- ☐ Should I stop or adjust any of my supplements?

About Monitoring & Follow-Up

- ☐ How do we measure whether treatment is working? (What should improve, and on what timeline?)
- ☐ Should I track symptoms? What tool or method do you recommend?
- ☐ How often should I follow up?
- ☐ When should I be concerned enough to seek urgent care?
- ☐ Should I be coordinating this care with my gastroenterologist?

About Your Specific Situation

- ☐ Given my UC, are there psychiatric medications that could worsen my gut condition?
- ☐ Could any of my supplements be causing or worsening the cognitive symptoms? (paradoxical effects, cholinergic overload, etc.)
- ☐ Is there a risk of serotonin syndrome given my supplement stack?
- ☐ Should I see a neurologist as well, or is psychiatry the right starting point?

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8. What to Expect at a First Psychiatric Consultation

A first appointment typically lasts **45–90 minutes** and covers:

1. **History taking:** They'll ask about your symptoms, when they started, what makes them better/worse
2. **Medical & psychiatric history:** Family history of mental illness, your previous diagnoses
3. **Substance & medication review:** Everything you take (this is why your list above matters)
4. **Mental status examination:** They'll assess your appearance, speech, mood, thought processes, cognition, insight
5. **Risk assessment:** They'll ask about self-harm, suicidal thoughts — answer honestly, this is standard
6. **Diagnosis discussion:** They may give a preliminary impression or want to gather more data (labs, etc.)
7. **Treatment plan:** Could include medication, therapy referral, lab orders, or referral to another specialist

> 💡 **Be honest about everything.** Supplement use, self-medication, substance use — a psychiatrist needs the full picture to help you safely. They've heard it all and won't be shocked.

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9. Practical Prep Checklist

- ☐ **Fill in all the `[...]` sections** above before the appointment
- ☐ **Write down your supplement list** — bring the actual bottles if possible
- ☐ **Bring recent lab results** (from your GI doctor, GP, or any recent bloodwork)
- ☐ **List current medications** from all doctors (GI, GP, etc.)
- ☐ **Note any recent major life events** or stressors
- ☐ **Write 2–3 specific examples** of cognitive failures (e.g., "Last Tuesday I couldn't finish a simple email I'd normally write in 5 minutes")
- ☐ **Bring a notebook** or use your phone to take notes during the appointment
- ☐ **Bring a trusted person** if you think you might forget what was discussed
- ☐ **Note any questions that come up** between now and the appointment

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10. Red Flags to Mention Immediately

If any of these apply, **tell the psychiatrist right away** — don't wait to be asked:

- ☐ **"Issues staying alive" — be explicit.** Does this mean:
  - Suicidal thoughts? (even fleeting — mention them)
  - Suicidal planning or attempts? (this is urgent — be direct)
  - Feeling like your body is failing / your health is life-threatening?
  - A sense of hopelessness about ever getting better?
  - **Whatever it means, spell it out clearly.** This is the single most important thing to communicate.
- ☐ Self-harm urges or behaviors
- ☐ Severe depression episodes
- ☐ Panic attacks
- ☐ Hallucinations or unusual perceptions
- ☐ Significant weight loss (from UC or otherwise)
- ☐ Inability to function at work or in relationships

- [ ] Rapid mood swings
- [ ] Any neurological symptoms (seizures, numbness, vision changes, severe headaches) — these warrant a neurology referral
- [ ] **7-year duration of serious health issues** — this is a long time. Make sure the doctor understands this isn't a recent development; it's chronic and you've been suffering for years

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> **Reminder:** You are the expert on your own experience. The psychiatrist is the expert on treatment. You're collaborating. It's okay to advocate for yourself, ask questions, push back gently, or request a second opinion. If you don't feel heard, that's valuable information too.

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11. Don't Forget These (Easy to Miss, Matters a Lot)

📞 Coordination Between Doctors

Your psychiatrist needs to talk to your gastroenterologist. Your symptoms cross both specialties. Ask:

- [ ] *"Can you coordinate with my gastroenterologist? My cognitive symptoms may be driven by my UC inflammation, and I don't want two doctors prescribing in isolation."*
- [ ] *"Who is the lead doctor managing my overall case — you, my GI, or my GP?"*

💊 B12 & Iron Injections (Critical for UC Patients)

Oral B12 and iron may not work if your gut is inflamed from UC — the absorption is broken. **Injections bypass the gut entirely.** This could be the fastest path to cognitive recovery if you're deficient.

- [ ] Ask directly: *"My gut may not absorb oral B12 or iron. Can I get these via injection?"*
- [ ] This alone could explain a significant portion of your cognitive symptoms — don't let it get overlooked

🧠 Neuropsychological Testing

If the psychiatrist isn't sure what's causing the cognitive deficits, a **neuropsychological assessment** can map exactly which cognitive domains are impaired (memory, executive function, processing speed, attention). This gives a baseline and helps track recovery.

- [ ] *"Can I get a referral for neuropsychological testing? I want to know exactly what's impaired and have a baseline to measure recovery against."*

😴 Sleep Architecture

Poor sleep destroys memory consolidation and neuroplasticity. If your UC symptoms are destroying your sleep, that alone could explain much of the cognitive decline.

- [ ] Tell the doctor: *"My sleep is disrupted by [UC symptoms / pain / anxiety / other]. How much of my cognitive decline could be from chronic sleep deprivation?"*
- [ ] Ask about a sleep study if sleep is seriously disrupted

🦠 Gut Microbiome Testing

Your food poisoning + UC may have caused significant dysbiosis. The gut microbiome directly produces neurotransmitters (90% of serotonin, much of your GABA) and communicates with the brain via the vagus nerve.

- [ ] *"Has my gut microbiome been tested? Could dysbiosis from the food poisoning be driving my brain symptoms?"*
- [ ] Ask about a **stool test** (calprotectin, microbiome analysis) if not done recently

🚨 Emergency Plan

If things get worse before the appointment:

- **116 123** — Slovenia free psychological support line (daily 19:00–24:00)
- **112** — Emergency (if in immediate danger or acute crisis)
- **Your GP** — Can fast-track a psychiatric referral if urgency increases
- Write your crisis plan here: *what will you do, who will you call, where will you go if things get critical before the appointment?*

📝 Bring Someone With You

After 7 years of cognitive decline, you may not remember everything the doctor says. Bring a trusted person to:

- Take notes
- Remind you of symptoms you wanted to mention
- Help you remember the treatment plan afterward
- If you can't bring someone, ask the doctor if you can **record the session** on your phone (with their permission)

⏱️ If the First Appointment Isn't Enough

First consultations are often 45–60 minutes — that may not be enough for a complex 7-year history.

- [ ] *"I know this is a lot. Can we schedule a follow-up soon to go deeper once the bloodwork comes back?"*
- [ ] Ask for a written summary of the appointment (diagnosis, plan, next steps)
- [ ] Don't leave without knowing: what's the next step, and when?